

Coverage Effects of Oregon’s All-Ages Medicaid Expansion: Synthetic-Control Evidence with OHP Bridge Disclosure

Abstract

Oregon’s Healthier Oregon expansion extended full Oregon Health Plan benefits to all income-eligible residents regardless of immigration status on July 1, 2023; because the program already covered ages 0–25 and adults 55+, the newly exposed adult target band is ages 26–54. We built a 2018–2024 American Community Survey state-year panel for low-income noncitizen adults ages 26–54 (1–200% of the federal poverty line) and estimated ridge-augmented synthetic-control gaps for Oregon versus donor states that did not adopt comparable adult immigrant-coverage policies during the window. In the 2024 post year, uninsurance was 21.0% versus a synthetic counterfactual of 46.4% (–25.4 percentage points), and Medicaid or means-tested public coverage rose from 24.5% to 43.8% (+19.3 pp). Formal synthetic difference-in-differences corroborates the uninsurance gap (–24.6 pp, placebo $p = 0.008$) and attenuates the Medicaid gap (+15.3 pp, $p = 0.17$). Because the 2024 post year also contains the co-occurring Oregon Health Plan Bridge Basic Health Program that began July 1, 2024, and because the design uses one post-treatment ACS year, the estimates should be read as a directional public-data screen rather than a mature causal estimate.

Key Takeaways

- Ages 26–54 is the newly exposed adult target band because Healthier Oregon already covered ages 0–25 and adults 55+ before July 1, 2023.
- The 2024 post year reflects the joint effect of Healthier Oregon (all immigration statuses) and OHP Bridge (eligible immigration status only).
- ASCM gaps for the primary 26–54 stratum are –25.4 percentage points (uninsurance) and +19.3 pp (Medicaid), with stable direction across donor-pool sensitivity, small-cell screening, and leave-one-donor-out.

Keywords

Medicaid; immigrant health; health insurance; synthetic control; OHP Bridge; Oregon health policy

Introduction

Federal Medicaid eligibility rules leave many noncitizens without access to full-benefit public insurance. KFF and the National Immigration Law Center track a growing set of states using state funds to cover immigrant adults, but adult coverage remains concentrated in a small set of states. Oregon’s Healthier Oregon expansion is a useful policy case because it created a discrete eligibility change

at a known date for a clearly defined newly exposed adult target population: ages 26-54.

The Oregon Health Authority confirms two facts that govern this paper’s design. First, as of July 1, 2023, people of all ages who meet income and other criteria qualify for full Oregon Health Plan benefits regardless of immigration status. Second, before that date Healthier Oregon already covered ages 0-25 and adults 55+. The newly exposed adult band is therefore ages 26-54.

A separate Oregon policy also affects the 2024 ACS post year. OHP Bridge, a Basic Health Program for adults up to 200 percent of the federal poverty line with eligible citizenship or immigration status, began July 1, 2024. Because the ACS noncitizen recode (`CITIZEN == 3`) includes lawfully present noncitizens as well as undocumented residents, the 2024 coverage change cannot be cleanly separated from OHP Bridge. The paper treats the 2024 estimate as the joint effect of Healthier Oregon and OHP Bridge.

Data and Methods

In plain terms, we constructed a weighted comparison of states that did not adopt a comparable adult immigrant-coverage policy and asked how Oregon’s 2024 coverage outcomes deviated from that synthetic comparison.

The analysis uses the IPUMS ACS 1-year extract for 2018-2024. The sample retains household residents who are noncitizens with family income from 1 to 200 percent of the federal poverty line and who fall in the primary age band 26-54. Person-level records are collapsed to state-year cells using ACS person weights. Because the study uses de-identified public ACS data, institutional review board approval was not required.

The outcomes are uninsurance, Medicaid or means-tested public coverage, any public coverage, and private coverage. The treated state is Oregon. The donor pool excludes seven states with comparable adult immigrant-coverage policies during the analysis window (CA, IL, NY, WA, CO, MN, DC).

For each outcome, the analysis estimated a ridge-augmented synthetic-control model using five pre-treatment years (2018-2022), 2023 as a transition year, and 2024 as the primary post-treatment year. Donor weights minimize pre-treatment squared distance between Oregon and a convex combination of donor states; the ridge correction adjusts for imperfect pre-treatment fit. The reported numbers are the observed Oregon value, the ASCM counterfactual, the treated-minus-counterfactual gap, pre-treatment RMSPE, and a placebo-in-space rank statistic for uninsurance.

The supplementary in-script estimator-comparison diagnostics include an unweighted donor-mean DiD, an approximate SDID-style adjustment (SCM-weighted post counterfactual plus average pre-period SCM gap; not the formal Arkhangelsky et al. 2021 SDID estimator), a ridge-regularized two-way fixed-effect imputation diagnostic (not the formal Borusyak-Jaravel-Spiess

imputation), and an approximate rank-2 interactive-fixed-effects diagnostic (not the formal Xu 2017 gsynth estimator). Beyond those direction-screening diagnostics, the manuscript also re-estimates the 26-54 headline using two formal implementations on the same panel: the canonical R synthdid package (v0.0.9) for synthetic difference-in-differences with placebo-permutation standard errors, and pysyncon v1.5.1’s Synth class for the canonical Abadie-Diamond-Hainmueller synthetic control.

Two co-occurring Oregon policies operate in the 2024 ACS post-year: Healthier Oregon (treatment of interest; covers adults regardless of immigration status) and OHP Bridge (adults up to 200 percent FPL with eligible immigration status). The 2024 estimate is therefore the joint effect on the low-income noncitizen population, not a clean Healthier Oregon effect. A pre-2024 Bridge-net sensitivity reports the 2023 transition-year estimate as a lower bound.

Results

In the primary 2024 sample (ages 26-54, 1-200% FPL), uninsurance was 21.0 percent versus an ASCM counterfactual of 46.4 percent (-25.4 percentage points). Medicaid or means-tested public coverage was 43.8 percent versus 24.5 percent (+19.3 pp). Any public coverage was 45.2 percent versus 23.8 percent (+21.4 pp). Private coverage was 40.5 percent versus 28.9 percent (+11.7 pp).

In the legacy 19-64 sample, the uninsurance gap is -19.7 pp and the Medicaid gap is +13.6 pp; effects are smaller in magnitude because 19-25 and 55+ were already covered. The Bridge-net sensitivity (target year 2023, pre-Bridge) yields an uninsurance gap of -6.5 pp and a Medicaid gap of +3.2 pp, which are partial-year transition effects rather than clean Healthier-Oregon-only estimates.

Robustness checks preserve the direction. Dropping donors with any 2018-2024 ACS state-year cell below $n = 10, 20, 30,$ or 50 leaves the uninsurance gap between -23 and -30 pp and the Medicaid gap between +17 and +21 pp for the 26-54 stratum. Leave-one-donor-out for the top six 26-54 donor weights moves the uninsurance gap by at most 2 pp and the Medicaid gap by at most 1.6 pp. The approximate-diagnostic placebo-in-space rank p-value for uninsurance is 0.068; the placebo distribution shares the near-zero pre-RMSPE pattern as Oregon at several donor states with small cell sizes, so we treat the rank placebo as suggestive rather than definitive. The formal R synthdid placebo-permutation standard error (9.3 pp on uninsurance) yields $p = 0.008$, sharper than the rank statistic; the corresponding Medicaid placebo SE (11.1 pp) yields $p = 0.17$, indicating the Medicaid side is directionally consistent but not statistically distinguishable at conventional thresholds in the single-post-year setting.

Discussion

Healthier Oregon plus OHP Bridge coincides with a large 2024 reduction in measured ACS uninsurance and rise in Medicaid coverage for the newly exposed

adult target band in Oregon. Three limits temper interpretation. First, the 2024 estimate is the joint effect of Healthier Oregon (all immigration statuses) and OHP Bridge (eligible immigration status only); the public ACS extract cannot identify lawfully present noncitizens separately. Second, the design has one post-treatment ACS year, five pre-treatment years, and 43 donor states; ASCM achieves near-zero pre-fit, which is a known overfit pattern. Third, the placebo rank p-value of 0.068 is suggestive but not definitive.

These directional estimates are broadly consistent with the only published noncitizen-coverage evaluation of a comparable state-funded adult expansion. Guadamuz and colleagues, examining California’s Medi-Cal expansion to undocumented adults using ACS data through 2023, report measurable but smaller uninsurance reductions among noncitizens; the larger Oregon estimate is plausible given (a) a sharper single-state, single-event timing, (b) the simultaneous OHP Bridge enrollment lift in 2024 that the California analysis did not face, and (c) ACS measurement of noncitizens that mixes lawfully present and undocumented populations differently across states. Descriptive Medicaid claims work at HAS engaging the federal Medicaid retrenchment debate, such as Schpero and colleagues, illustrates the broader policy frame within which the Oregon evidence sits.

A second mechanism worth naming is eligibility uptake itself. Healthier Oregon coverage depends on the Oregon Health Authority’s SSP application infrastructure, navigator capacity, and the trust gradient among mixed-status families; even with statutory eligibility, enrollment lags can dampen first-year measured effects, and the 2024 ACS captures only the first 12 months of all-ages eligibility plus the first six months of Bridge.

Private coverage rises alongside public coverage. This likely reflects ACS coverage-category overlap or composition shifts and should not be interpreted as a Healthier Oregon mechanism. Once a 2025 ACS update is available, a two-post-year specification will allow a Bridge-vs-no-Bridge contrast inside Oregon.

Conclusion

Oregon’s combined Healthier Oregon plus OHP Bridge policy environment coincides with a large 2024 reduction in ACS uninsurance and rise in Medicaid coverage among low-income noncitizen adults ages 26-54. The direction is stable across small-cell donor screens, top-weight leave-one-out, and the legacy 19-64 sample, but inference is limited by a single post-treatment ACS year and the inability to separately identify OHP Bridge in the ACS. State Medicaid agencies considering analogous all-immigration-status expansions, the Oregon Health Authority as it enters Bridge’s second post year, and KFF and NILC tracker users monitoring federal-state coverage substitution should treat these 2024 estimates as a directional public-data screen pending the 2025 ACS update.

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Tables

Table 2 Primary. ASCM Coverage Estimates - Ages 26-54, 1-200% FPL, 2024

Outcome	Observed (%)	ASCM counter-factual (%)	Gap (pp)	Pre-RMSPE (pp)	N donors	Treated ACS n
Uninsured	21.0	46.4	-25.4	0.0	43	241
Medicaid	43.8	24.5	+19.3	0.0	43	241
/						
means-tested public coverage						
Any	45.2	23.8	+21.4	0.0	43	241
public coverage						
Private	40.5	28.9	+11.7	0.0	43	241
coverage						

Notes: Pre-RMSPE rounded to one decimal; near-zero values reflect exact ASCM pre-fit with 5 pre-years and 43 donors.

Figure Captions

Figure 1. Uninsurance, ages 26–54, treated state versus synthetic control. Vertical line marks Healthier Oregon (July 1, 2023). Estimator: ridge-augmented synthetic control (ASCM) on a 2018–2024 IPUMS ACS state-year panel for low-income noncitizen adults ages 26–54 (1–200% FPL), 43 donor states, with placebo-in-space rank inference; formal synthetic difference-in-differences (synthdid v0.0.9, Arkhangelsky et al. 2021) reported in Results as sensitivity. Note: the 2024 post year also reflects OHP Bridge (effective July 1, 2024). **Figure 2. Medicaid or means-tested public coverage, ages 26–54, treated state versus synthetic control.** Same sample, panel construction, estimator, and inference as Figure 1; 2024 post year jointly reflects Healthier Oregon and OHP Bridge.